**جوستاف روسي انترناشيونال - مصر (EPE) ***

# طلب علاج أول مرة / متردد

**إسم المريض رباعياً :** سيده عبدالفتاح علي علي
**المحافظة :** القاهره **الحي / المدينة :** الساحل **العنوان :** *
**رقم البطاقة :** 26011220104761 **رقم التليفون :** 01227386617 **تاريخ تسجيل الإستمارة :** 11-10-2026
**العمر(تاريخ الميلاد) :** 22-11-1960 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

Name: سيده عبدالفتاح علي علي
National ID: 26011220104761

## تقرير طبي

السيد

تحية طيبة وبعد,,

| | |
|---|---|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم: | رقم طبي: |

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

| | |
|---|---|
| تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd | تاريخ وساعة الخروج : PM 01:17 05-07-2026 |

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

| | |
|---|---|
| التاريخ : 05-07-2026 | الوقت : -- --:-- |
| | الطبيب المعالج : د/شيماء لاشين |




National ID – Front
[[DOCTYPE:ID_FRONT]]

٢٩٠١١٢٢٠١٠٤٧٦١ ٢٠٢٣/٠٤

ربة منزل

أنثى مسلمة أرملة

١ البطاقة سارية حتى ٢٠٣٠/٠٤/٠٣



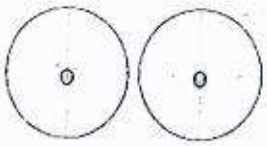
National ID – Back
[[DOCTYPE:ID_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

# MDT form

A-Patient ID:
1- Name : سيده عبدالفتاح على على
Sex: Female
3- Medical number: 83258534
Old Med : 350-2025
2-ID number: 26011220104761
Patient Age : 64 Y



| | | | | | | | |
|---|---|---|---|---|---|---|---|
| Family history | Family history: -ve | Phone | Phone: 01227386017 | BMI | BMI: 40 | REASON OF MDT | REASON OF MDT: POST OPERATIVE |
| marital status | Marital Status: MARRIED | NRS Score | NRS Score: 1 | GROUP | GROUP: C | Date of MDT | Date of MDT: 2025-05-22 |
| Co morbidity | Co Morbidity: -ve | Menopausal status | Menopausal Status: POSTMENOPAUSAL | Initial STAGE | Initial STAGE | | |

## Clinical data

**Breast Imaging :**
rt breast mass+rt axillary LN

**Mets Workup :**
free

| BX | ER | PR | Her 2 Neu | SISH | Ki67 |
|---|---|---|---|---|---|
| BX COMMENT: invasive mammary carcinoma | ER: 8/8<br>ER COMMENT: | PR: 7/8<br>PR COMMENT: | Her 2 Neu COMMENT: -ve | SISH COMMENT: | 10 |

| T | N | M |
|---|---|---|
| CT2 | 1 | 0 |

**Sonomammography**
RT UOQ10 OCLOCK MASS 1.6CM, PATH LEVEL I AXILLA 3 LNS

**PATIENT HISTORY**
upfront RT BCS+ALND: bifocal ILC GII with lobular neoplasia , focal DCIS PT3(6CM), N3(21/21) with extra capsular extension with diffuse infiltration of axillary fibro fatty tissue deep & superior margins showed DCIS
free extra deep & sup. Margins
free all other margins

## MDT OUTCOME & PLAN

MDT OUTCOME & PLAN:
FOR ADJUVANT ACX4 CYCLES THEN TAXANESX4
PORT
ADJUVANT AI
ZOMETA/6MONTHS

## SIGNATURE

| | | | |
|---|---|---|---|
| MEDICAL ONCOLOGY | MEDICAL ONCOLOGY: د/ جميل | RADIOLOGY | RADIOLOGY: د/ هاله البدري |
| SURGERY | SURGERY | PATHOLOGY | PATHOLOGY |
| Psychiatric | Psychiatric | Nutrition | Nutrition |

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / سيده عبدالفتاح علي علي | Request No. | : 13006378 |
| Patient No. | : 26011220104761 | Request Date | : 23/01/2025 |
| Path No. | : SR-139/25 | Verification Date | : 20/02/2025 |
| Sex | : Female | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age | : 64 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical Data: Right breast cancer.
Nature of Specimen: Slide Revision.

Gross Examination:
- One paraffin block labeled by (13-25) was recieved.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive mammary carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis):
Grade II (3+2+1).
* LCIS: Focal.
* Angioinvasion: Not detected.
* Perineural invasion: Not detected.
* Microcalcifications: Not detected.
* IPT:
- E-cadherin: Negative
- ER: Strong nuclear reaction in 85% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 7/8).
- Her2: No membran reaction in tumor cells (score 0)
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:
Right breast cancer, Slide revision:
Invasive lobular carcinoma, Grade II with focal LCIS.
IPT:

Verified by :
Dr.Toka Medhat



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / سيده عبدالفتاح علي علي | Request No. | : 13006378 |
| Patient No. | : 26011220104761 | Request Date | : 23/01/2025 |
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| Age | : 64 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : .marwa mohamed youss |

## Pathology Report

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0)
- Ki67: Positive in 10% of tumor cells

Signature
Prof. Dr. Lobna Omar
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

| | | | |
|---|---|---|---|
| Patient Name | :الأستاذه / سيده عبدالفتاح علي علي | Request No. | : 13007255 |
| Patient No. | :26011220104761 | Request Date | : 15/04/2025 |
| Path No. | : 1332/25 | Verification Date | :29/04/2025 |
| Sex | : Female | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age | :64 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : Mrs.Mayada Hegazy |

## Pathology Report

Clinical Data : Right breast cancer
Nature of the specimen : Right BCS & Right ALN biopsy

Gross Examination:
- Procedure: Right BCS specimen measured 7x4x4 cm , covering skin ellipse measured 6 x 2.5 cm .
- Tumor extent : A well defined greyish white firm mass measured 6x3x2 cm .
- Distance from margins: 0.2 cm away from superior margin , 0.1 cm away from deep margin .
- Lymph nodes number and size: A fibrofatty tissue piece measured 7x7x7 cm enclosing 21 ALNs up to 5x3x3 cm .
- Extra (superior &deep ) margin : Fibrofatty tissue piece measured 6x5x1.5 cm , free of gross induration
- Frozen section: Superior &deep margins positive for DCIS ,Free extramargins , Free other margins

Microscopic examination:
- Histological type:Bifocal Invasive lobular carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Angioinvasion: Positive
- Ductal carcinoma in situ: present
- Lobular insitu : Positive
- Lymph node involvement: Positive ALNs (21/21), with extra capsular extension , (It contained axillary breast tissue showing focus of (1x1 cm of invasive lobular carcinoma grade II )
- Resection Margins: Deep margin showed DCIS, Free extra superior & deep margin , Free other margins
- Additional pathologic findings: Microcalcification

Verified by :
Dr.Heba Hasan



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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / سيده عبدالفتاح علي علي | Request No. | : 13007255 |
| Patient No. | : 26011220104761 | Request Date | : 15/04/2025 |
| Path No. | : 1332/25 | Verification Date | : 29/04/2025 |
| Sex | : Female | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age | : 64 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : Mrs.Mayada Hegazy |

## Pathology Report

Diagnosis:
Right BCS & Right ALN biopsy:
Bifocal Invasive lobular carcinoma Grade II with lobular neoplasia, focal DCIS
- Positive LNs (21/21) with extracapsular extension with diffuse extension infitration of axillary fibrofatty tissue
- Deep &superior margins showed DCIS, Free extra deep &superior margins, Free other margins
- Stage : pT3 , pN3

Prof. Dr. Gamal Abdelatty
Dr.Menna Nabil

Verified by :
Dr.Heba Hasan



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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
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Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

| | |
|---|---|
| Patient Name: Sayeda Abd Al Fattah Ali Ali | Date : 13/02/2025 |
| Exam : C.T CHEST | Age : 64 Year(s) |
| Medical ID: 350/2025 | Pat. ID: 97398 |

# NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

## A reported case of breast cancer, Findings:

## I Chest: -

- Bilateral breast asymmetrical parenchymal condenstaiions more at the right, For SM correlation.
- Right axillary enlarged lymph nodes, the largest measures 29x14 mm.
- Prominent left axillary lymph nodes
- Bilateral few scattered variable sized pulmonary nodules, the largest is seen at the right upper lung lobe apical segment and measures about 5mm.
- No hilar or mediastinal grossly enlarged lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

## II Abdomen & pelvis:

- The liver is seen mildly enlarged with homogenous low parenchymal CT texture. No definte focal lesions (on non contrast base) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes( apart from bilateral few simple renal cystes, the largest at the left measures 23x19mm).
- GB few tiny dense stones.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- underfilling of the urinary bladder with no filling defects or diverticular outpouching, few air foci are seen inside.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Tiny metallic foreign body is note within the left pelvoc cavity , for clinical correlation.

## *Scanned skeleton:

- Few sclertoic tiny foci seen at D9 and L2 vertebral bodies .
- Otherwise, no definite worrisome lytic or sclerotic focal lesions.

CT Scan
[[DOCTYPE:CT_SCAN]]

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Radiology Department

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

| | |
|---|---|
| Patient Name: Sayeda Abd Al Fattah Ali Ali | Date : 13/02/2025 |
| Exam : C.T CHEST | Age : 64 Year(s) |
| Medical ID: 350/2025 | Pat. ID: 97398 |

## Opinion:

**A reported case of breast cancer showing,**

- **Bilateral few scattered variable sized pulmonary nodules for follow up.**
- **No evidence of hepatic metastatic deposits on non-contrast basis.**

**DR/ Maie Khattab,MSc,**

*Dr. Walid Mohamed, M.D.*

CT Scan
[[DOCTYPE:CT_SCAN]]

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

| | |
|---|---|
| Patient Name: Sayeda Abd Al Fattah Ali Ali | Date : 06/03/2025 |
| Exam : ultrasound BREAST (BOTH SIDE ) | Age : 64 Year(s) |
| Medical ID: 350/2025 | Pat. ID: 97398 |

## Combined bilateral breast mammography and complementary ultrasound

**Indication:**

Right breast mass pathologically proven ILC.

**Technique**

- Bilateral MLO and CC views
- Bilateral breast ultrasound

**Report:**

- Scattered fibro glandular density (ACR B)
- Right breast shows UOQ distortion with skin retraction.
- Complementary ultrasound revealed right breast irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 10 o'clock position 4cm from the nipple, 0.9cm from the overlying skin, measuring 1x1.6cm, consistent to the palpable lump.
- No left breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological right axillary level I lymph nodes, about 3 in number, the largest is measuring 3.1cm, with cortical thickness reaching 6mm.
- Free right infra and supra-clavicular regions
- Nonspecific right axillary LN.

**Opinion:**

- Right breast UOQ distortion, pathologically proven malignant (BIRADS 6)
- Pathological right axillary LNs.
- Benign findings left breast (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

06/03/2025 2:41 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

*Nuclear Medicine Unit* وحدة الطب النووى

*Sayeda Abdel Fattah Ali* *Date: 06/03/2025*

## Tc99m MDP SKELETAL SCINTIGRAPHY

**History**

- A64-year-old lady withhistory ofbreast cancer.

**Technique**

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

**Findings**

- Warm tracer uptake is noted at lower dorsal and upper lumbar spine.
- Arthritic changes are noticed at large joints.
- Otherwise, the rest of the scanned skeleton is unremarkable.

**IMPRESSION**

- No convincingscintigraphic evidence of osseous metastases identified.
- Benign looking tracer uptake pattern confined to spine as described, likely degenerative, for regional radiological correlation and follow up.

**Appreciated,**

**Dr.Hossam Sekkina, M.Sc.**

**Dr. Alaa Nagi, MD**
**Nuclear Medicine**



[[DOCTYPE:BONE_SCAN]]

Nssr El Islam hospital
Ultrasound
Tel : 24328495 - 24328496

مستشفى نصر الاسلام العام
وحدة الموجات
ت : ٢٤٣٢٨٤٩٦ - ٢٤٣٢٨٤٩٥

Name: - Sayeda Abdelfattah
Date: - 30 December 2024

# *Both breasts ultrasound study*

*Skin, nipple, and areola:*

- *No diffuse or localized lesions could be detected bilaterally*

*Parenchyma:*

- *Right sided ill-defined hypoechoic speculated mass lesion seen at the upper lateral quadrant measuring gabout 20 x 17 mm (highly suspicious of malignancy) BIRADS 4c*
- *Otherwise, the rest of both breasts showing mixture of hypoechoic fatty, echogenic fibro glandular tissues with predominate fatty component, with no other nodules, masses, cysts or ductectasia bilaterally*

*Axillae:*

- *Few right sided mildly enlarged LNs (all with preserved hilar fat) the largest 30 x15 mm.*

## *Impression:*

- *Right breast highly suspicious mass lesion as described.*

*Best regards,*
*Dr. / Hussein Abdelbary.*
*Consultant radiology.*



Sonography
[[DOCTYPE:SONOGRAPHY]]

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مستشفى أورام دار السلام
ادارة الاشعة

| | |
|---|---|
| Patient Name: Sayeda Abd Al Fattah Ali Ali | Date : 03/02/2025 |
| Exam : ultrasound ABDOMEN& PELVIS | Age : 64 Year(s) |
| Medical ID: 350/2025 | Pat. ID: 97398 |

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:**

***Liver: -***

- *enlarged in size (17cm at Rt MCL) bright parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

***Gall Bladder: -***

- Average distention and capacity with multiple small stones seen inside, *the wall is not thickened.*
- *CBD is not dilated.*

***Spleen: -***

*Average size, homogenous.*

- **Kidneys: -**
- *Both are average in size,shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *Left lower zone cortical cyst 18mm*

***Para-aortic and pancreatic area****: show no gross abnormalities.*

***No ascites***

***Urinary Bladder: -***

*Not distended enough*

*Opinion:*

***Bright hepatomegaly likely fatty***
***Calcular GB***
***Left renal cortical cyst***
***Colonic gaseous distention***

Signature Dr . Nermien Ramadan

03/02/2025 3:01 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]